NCCP 00271 · VERSION 6

Your treatment, explained

Extensive-stage small-cell lung cancer · chemotherapy · NCCP 00271 v6 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Carboplatin + Etoposide

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe diarrhoea, vomiting despite medicines, inability to drink, very little urine, or a spreading/blistering rash. Tell staff immediately about wheeze, swelling, dizziness or faintness during an infusion. Report hearing changes, ringing in the ears, numbness, weakness or difficulty walking promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Three treatment days in a 21-day cycle
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Your treatment and its aim
- Before treatment
- Your visits and home plan
- Effects to discuss and monitor
- When to seek urgent help
- Review and the next plan
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

Your two chemotherapy medicines

Carboplatin + Etoposide is chemotherapy used for extensive-stage small-cell lung cancer. It aims to control the cancer and its symptoms. Ask your oncology team what benefit is realistic for you and how this option compares with other suitable treatments.

PLATINUM CHEMOTHERAPY

Carboplatin



Forms links that damage DNA

Carboplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing; normal tissues can also be affected. Carboplatin dosing is tailored using a verified kidney-function result; low blood counts can be an important limiting effect.

Platinum enters cells → DNA links form → DNA copying disrupted

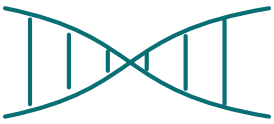
What to watch for

Report changes in urine, hearing, ringing in the ears, numbness or daily function. Tell the team about infection, unusual bruising or bleeding and any symptoms during the infusion.

Sources: nccp · platinum

TOPOISOMERASE II CHEMOTHERAPY

Etoposide



Disrupts DNA repair during cell division

Etoposide interferes with topoisomerase II, an enzyme that helps manage DNA during copying. DNA breaks can remain unrepaired and cancer cells may stop dividing or die. Healthy cells are also affected; the infusion or prescribed capsules deliver the same medicine by different routes.

Topoisomerase II disrupted → DNA breaks persist → Cell division interrupted

What to watch for

Report fever, bleeding, mouth sores or diarrhoea. Tell the nurse about faintness, rash or breathing difficulty during an infusion. If capsules are prescribed, use the exact pharmacy instructions and contact the team about a missed or vomited dose.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Three treatment days in a 21-day cycle

THREE TREATMENT DAYS, THEN RECOVERY

Day 1 platinum and etoposide; Days 2–3 etoposide



Carboplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

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Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Checks through the whole course

The columns show the order of care, not equal time intervals. Extra tests depend on symptoms, results and the consultant's plan.

Check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Blood counts	Baseline blood count	Review results before treatment	Blood count weekly before treatment	Review infection, bleeding and previous toxicity	Follow unresolved low counts
Kidneys, liver and fluids	Renal/liver profile; verify clearance and dose suitability	Renal/liver tests before each cycle; verify current carboplatin GFR	Repeat sooner after dehydration or illness	Review organ function before continuation or modification	Follow kidney/ electrolyte/liver abnormalities
Carboplatin dose verification	Measured GFR preferred or validated estimate	If estimated: creatinine within 48 hours of dosing	Reassess if health or kidney function changes	If creatinine >110% of isotope-baseline value, reassess GFR	A future regimen requires its own dose review
Hearing and nerves	Record hearing/nerve symptoms and previous platinum exposure	Regular hearing and neurological review	Report tinnitus, hearing loss, numbness or loss of function	Balance benefit and lasting toxicity	Review persistent changes

Check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Prevention and route	Confirm infusion or capsule plan and a written multi-day anti-sickness schedule	Check support before each treatment day; pharmacy confirms oral doses	Follow support through two days after last SACT; do not extend medicines yourself	Review interactions and support after any dose/route change	Confirm which support medicines stop or continue
Disease assessment	Confirm extensive-stage SCLC and individual treatment aim	Assess symptoms and fitness	Extra assessment for new concerns	Scans/tests as directed by consultant; no universal scan interval	Agree follow-up or a separate next treatment plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00271 v6 · official regimen · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance · Manufacturer carboplatin product information

THE WHOLE TREATMENT JOURNEY

Carboplatin + Etoposide: treatment and checks

Extensive-stage small-cell lung cancer · chemotherapy

Carboplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Chemotherapy course NCCP 00271	Agree indication, route and course	Day 1 Carboplatin + etoposide; etoposide continues Days 2–3	Recovery and symptom monitoring	The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.	Follow-up or a separately prescribed next plan
Blood counts NCCP 00271	Baseline blood count	Review results before treatment	Blood count weekly before treatment	Review infection, bleeding and previous toxicity	Follow unresolved low counts
Kidneys, liver and fluids NCCP 00271	Renal/liver profile; verify clearance and dose suitability	Renal/liver tests before each cycle; verify current carboplatin GFR	Repeat sooner after dehydration or illness	Review organ function before continuation or modification	Follow kidney/ electrolyte/liver abnormalities
Carboplatin dose verification NCCP 00271	Measured GFR preferred or validated estimate	If estimated: creatinine within 48 hours of dosing	Reassess if health or kidney function changes	If creatinine >110% of isotope-baseline value, reassess GFR	A future regimen requires its own dose review

Care / check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Hearing and nerves NCCP 00271	Record hearing/nerve symptoms and previous platinum exposure	Regular hearing and neurological review	Report tinnitus, hearing loss, numbness or loss of function	Balance benefit and lasting toxicity	Review persistent changes
Prevention and route NCCP 00271 · ANTIEMETIC	Confirm infusion or capsule plan and a written multi-day anti-sickness schedule	Check support before each treatment day; pharmacy confirms oral doses	Follow support through two days after last SACT; do not extend medicines yourself	Review interactions and support after any dose/route change	Confirm which support medicines stop or continue
Disease assessment NCCP 00271	Confirm extensive-stage SCLC and individual treatment aim	Assess symptoms and fitness	Extra assessment for new concerns	Scans/tests as directed by consultant; no universal scan interval	Agree follow-up or a separate next treatment plan

What each check is for

1

Before first treatment

Before dose authorisation

Confirm suitability and route.

Ask / act: Confirm the complete plan with the consultant and pharmacy.

Sources: NCCP 00271

2

Across Days 1–3

Every treatment day

The active medicines and route change within each cycle.

Ask / act: Confirm dose, route and today’s supportive medicines.

Sources: NCCP 00271

3

After the last treatment day

Two days after multi-day SACT

Prevention extends beyond the final dose.

Ask / act: Provide an individual written schedule and rescue instructions.

Sources: ANTIEMETIC

4

Course and response review

During and at the end of planned chemotherapy

Benefit and toxicity guide the next step.

Ask / act: Confirm completion, persistent-toxicity follow-up and any separate next regimen.

Sources: NCCP 00271

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.28

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Carboplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Carboplatin Carboplatin on Day 1 after clinical review	Day 1																				
Etoposide	Infusion Days 1–3, or prescribed Day 1 infusion / Days 2–3 capsules																				
Prevention medicines	Follow the individually reconciled multi-day plan																				
Report symptoms	Symptoms may occur at any time; this bar does not measure risk																				
Next-cycle review																				Tests and review before deciding the next cycle; appointments may differ	

Before the first dose: Confirm the diagnosis, treatment aim, blood tests, kidney/liver function and selected etoposide route.

Support after the last dose: The multi-day sickness-prevention plan needs to account for two days after the last SACT day. Take only the written prescribed course.

During the three treatment days: Day 1 includes both medicines; Days 2–3 continue etoposide by the prescribed route.

Across and after the course: Counts, kidneys, hearing and nerves need review throughout treatment. Some problems can persist after the final dose.

> **Why these bands stay broad:** This shows one reference cycle and the order of care. It does not predict a symptom-free day or when blood counts will fall. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00271 v6 · official regimen ↗ · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance ↗

> **There are no guaranteed "safe days".** Follow your unit's temperature threshold and emergency instructions. Sudden illness or shaking chills needs urgent advice even without a high temperature.

Read the whole cycle as text

Day 1: Day 1 in the unit

Carboplatin is infused over about 30 minutes, followed by etoposide over about one hour on Day 1. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation. Tell staff immediately about rash, swelling, wheeze, faintness or pain near the drip.

Day 2–3: Etoposide continues

Return for etoposide infusions unless the team prescribed the oral alternative. If you have capsules, follow the twice-daily pharmacy label and empty-stomach instructions. Confirm today's anti-sickness medicines. Contact the team about a missed or vomited capsule dose.

Day 4–5: After the last treatment day

Follow the written prevention and rescue plan. Multi-day antiemetic cover extends to two days after the final treatment day; the exact medicines are chosen by your team. Call for sickness despite medicines, inability to drink or reduced urine.

Day 6–20: Recovery and symptom monitoring

Fatigue, low counts, infection, mouth or bowel symptoms can occur between visits. Hearing and nerve symptoms may build across cycles. Use the urgent contact plan at any time; do not wait for the next appointment.

Day 21: Review before the next cycle

The team reviews blood counts, kidney/liver function and symptoms before deciding to continue, delay or change the plan. Bring your medicine list and symptom notes; confirm the remaining course and next review.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before each visit

Confirm transport for Days 1–3, blood-test arrangements, prescriptions and the 24-hour contact details. Tell the team if you are ill before travelling.

STEP 2

Time in the unit

Carboplatin is infused over about 30 minutes, followed by etoposide over about one hour on Day 1. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation.

STEP 3

Infusion access

Treatment may use a cannula or an existing suitable central line. Ask for line-care instructions if you have a line. There is no routine take-home infusion pump.

STEP 4

Capsules and home medicines

If prescribed, confirm your etoposide capsule doses, empty-stomach timing, safe handling and what to do if a dose is missed or vomited. Keep prevention and rescue medicines clearly labelled.

STEP 5

Support between visits

Arrange practical help if needed and keep symptom notes. Bring questions and a current medication list to review.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00271 v6 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Preventing sickness across all treatment days

Take the written anti-sickness plan even if you feel well. The plan must cover the risk of the medicines on each of Days 1–3 and the two days after the last treatment. Ask the pharmacist to show which medicines are scheduled and which are rescue medicines.

2 One clear steroid plan

The anti-sickness plan may include dexamethasone. Follow the prescribed day-by-day instructions; do not add an extra steroid course or extend it yourself. Tell the team about diabetes, poor sleep or mood changes.

3 Fluids and kidney protection

Ask for a personal drinking plan. Kidney results help calculate the carboplatin dose; illness or dehydration may require repeat tests. Call if you cannot drink or pass much urine.

4 If etoposide capsules are prescribed

The source alternative uses capsules on an empty stomach twice daily on Days 2 and 3. Your pharmacy label gives your individual capsule dose. Do not substitute capsules for an infusion yourself, crush/open capsules, double a missed dose or repeat a vomited dose without advice. Obtain safe handling and storage instructions.

5 Medicines, mouth care and infection advice

Check prescriptions, painkillers, supplements and vaccines with the oncology team. Some medicines affect kidney/hearing toxicity or etoposide exposure. Follow mouth-care advice and keep the unit's urgent contact instructions available.

Sources: nccp · antiemetic · hse · smpc

TREATMENT REFERENCE · 01

Your treatment and its aim

Confirm the full plan before starting.

The treatment covered here

This summary is for chemotherapy for extensive-stage small-cell lung cancer. Treatment that also includes immunotherapy or radiotherapy needs its own complete plan.

Sources: nccp

Why two medicines?

The platinum drug damages DNA and etoposide interferes with DNA repair. Using them together attacks cell growth in different ways. Healthy cells can also be affected.

Sources: nccp · smpc · platinum

Evidence and choices

Platinum and etoposide are established chemotherapy components for extensive-stage SCLC. A pooled comparison found no clear survival difference between carboplatin-based and cisplatin-based treatment, but their side effects differed. The elderly/poor-risk JCOG9702 trial used etoposide 80 mg/m² and split cisplatin, so its survival figures cannot be assigned directly to this 100 mg/m² etoposide schedule. These older chemotherapy comparisons do not rank current immunotherapy combinations or predict an individual outcome.

Sources: cocis · jcog9702

TREATMENT REFERENCE · 02

Before treatment

Preparation helps the team choose a suitable plan.

Tests and fitness

Blood counts, kidney and liver tests are checked before treatment. Tell the team about existing nerve/hearing problems, other medicines and any recent infection.

Sources: nccp

Route and practical arrangements

Confirm whether all three etoposide doses are infusions or whether you have capsules on Days 2 and 3. Do not substitute one route yourself.

Sources: nccp

Fertility, contraception and pregnancy

Discuss fertility preservation and contraception before treatment. Pregnancy and breastfeeding are exclusions. Ask for medicine-specific advice and the required contraception period.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Your visits and home plan

Three treatment days form one cycle.

Day 1

Carboplatin is infused over about 30 minutes, followed by etoposide over about one hour on Day 1. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation.

Sources: nccp

Days 2 and 3

Continue etoposide by the prescribed route, with risk-appropriate anti-sickness prevention each treatment day.

Sources: nccp · antiemetic

The following days

Continue only the written prevention schedule and monitor symptoms. The two days after the final treatment day are part of the multi-day antiemetic review.

Sources: antiemetic

TREATMENT REFERENCE · 04

Effects to discuss and monitor

Report problems early; they are not a measure of benefit.

Blood counts and infection

Both medicines may lower blood counts. Infection can become serious quickly; follow the urgent advice plan for fever, chills or sudden illness.

Sources: nccp · hse

Hearing, nerves and kidneys

Platinum medicines may affect hearing, nerves and kidneys, and some changes can last. Tell the team promptly about tinnitus, hearing change, numbness or reduced urine.

Sources: nccp

Mouth, bowel, hair and fatigue

Mouth soreness, nausea, bowel changes, hair loss and tiredness can affect daily life. Ask for help with symptoms; call urgently if you cannot drink or symptoms are severe.

Sources: smpc · hse

Later effects

Etoposide can be associated with a later blood cancer. Discuss serious and lasting risks together with likely benefit and alternatives.

Sources: smpc

TREATMENT REFERENCE · 05

When to seek urgent help

Use your unit’s 24-hour number and emergency instructions.

Urgent symptoms

Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe diarrhoea, vomiting despite medicines, inability to drink, very little urine, or a spreading/blistering rash. Tell staff immediately about wheeze, swelling, dizziness or faintness during an infusion. Report hearing changes, ringing in the ears, numbness, weakness or difficulty walking promptly.

Sources: nccp · hse

When calling

Give your regimen, last treatment day and symptoms. Explain whether you can drink and pass urine. Do not wait for the next appointment or for a high temperature if you feel very unwell.

Sources: hse

TREATMENT REFERENCE · 06

Review and the next plan

Completion and follow-up depend on the treatment aim and response.

Course review

The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

Sources: nccp

Disease assessment

Scans and other assessments are arranged by the consultant. Symptoms alone do not show whether treatment is working.

Sources: nccp

After the last dose

Review ongoing kidney, hearing, nerve and blood-count problems. Confirm which support medicines finish and agree follow-up. Any next treatment, including immunotherapy or radiotherapy, needs a separate plan.

Sources: nccp

Evidence and your own treatment decision

The exact medicines, dose, population and treatment era matter.

Platinum and etoposide are established chemotherapy components for extensive-stage SCLC. A pooled comparison found no clear survival difference between carboplatin-based and cisplatin-based treatment, but their side effects differed. The elderly/poor-risk JCOG9702 trial used etoposide 80 mg/m² and split cisplatin, so its survival figures cannot be assigned directly to this 100 mg/m² etoposide schedule. These older chemotherapy comparisons do not rank current immunotherapy combinations or predict an individual outcome.

Sources: cocis · jcog9702

PREPARE FOR THE CONVERSATION

Questions worth asking

Why are there three treatment days?

Etoposide is planned on three consecutive days. Usually you attend for infusions on all three. The alternative capsule plan is only used when prescribed and clearly labelled by your team.

Are etoposide capsules the same dose as the drip?

No. The source oral alternative has a different dose and twice-daily schedule. Your prescription and pharmacy label specify your individual dose and capsule strengths. Do not convert it yourself.

Will I have radiotherapy or immunotherapy too?

This guide covers Carboplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

How long does the course last?

The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier. The passport pages are reusable records, not a prescription for a fixed number of visits.

Do more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be assessed and treated; your team reviews benefit with the agreed clinical and scan plan.

Why might treatment be delayed or changed?

Blood counts, kidney/liver function or symptoms may need time to recover. A dose may change to reduce lasting harm. Ask which medicine changes, why, and when it will be reviewed.

What should I say when I call?

Give the regimen name, last treatment day, symptoms and when they started. Explain whether you can drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect information.

Can this guide decide whether treatment is safe?

No. Treatment decisions need your clinical results, symptoms, previous toxicity and prescriber/pharmacy review. This guide helps you understand and discuss the plan.

What should I be able to explain before I leave?

Ask your nurse or doctor to check that you can explain which medicines you are receiving, the treatment days, which medicines to take at home, and which number to ring urgently. Ask for the plan in writing if any part is unclear.

> **Try explaining the plan back:** Can you describe which days you have treatment, whether you have etoposide capsules, how you take prevention medicines and which number to call at any hour?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00271 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00271 v6 · official regimen
- Manufacturer etoposide product information: mechanism, risks and precautions
- HSE chemotherapy side effects and urgent advice
- NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance
- COCIS: individual-patient meta-analysis of first-line platinum chemotherapy in SCLC
- JCOG9702: carboplatin/etoposide versus split cisplatin/etoposide in elderly or poor-risk extensive SCLC
- HPRA Vepesid capsules: patient leaflet
- Manufacturer carboplatin product information

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6931/271_CarboplatinAUC5Etop100.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://www.medicines.org.uk/emc/product/3787/smpc

https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/United_Kingdom/Etoposide-spc-gb.pdf

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://pubmed.ncbi.nlm.nih.gov/22473169/

https://www.nature.com/articles/6603810

https://assets.hpra.ie/products/Human/30581/94054f4b-91cf-4c49-8063-040ead5c4a92.pdf